



**NHID-Clinical**

PAYER & PROCUREMENT REFERENCE

# Operational Blueprint

NHID-Clinical v1.3 · From RFP Language to Audit-Ready Production

---

Version 1.3 · June 2026 · CC BY 4.0

Author: Brianna Baynard · Independent, practitioner-led

NIST Public Comment: NIST-2025-0035-0026

**OPEN PROPOSAL — NOT A STANDARD OR REGULATORY REQUIREMENT**

NHID-Clinical is an early-stage open proposal by Brianna Baynard. It is a voluntary open proposal — not an accredited standard, not a certification program, and not a regulatory requirement. No HIPAA compliance, security guarantees, or liability protection are implied.

## EXECUTIVE SUMMARY

|                            |                                                                                                                                                                                     |
|----------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>What it is</b>          | A payer- and procurement-facing blueprint for turning NHID-Clinical's voluntary transparency controls into RFP language and audit-ready vendor accountability.                      |
| <b>How a payer uses it</b> | Add one standard conformance clause to a voice-AI vendor RFP or BAA amendment, have the vendor run the open test suite, validate the JSON traces yourself, then measure and decide. |
| <b>Operational impact</b>  | Machine-auditable proof of how each voice agent handles identity disclosure — no custom contract language, no internal validation tooling, and no production risk.                  |

|                      |                        |                      |                      |
|----------------------|------------------------|----------------------|----------------------|
| <b>5</b><br>Controls | <b>18</b><br>CTS Cases | <b>1160</b><br>Tests | <b>6</b><br>Adapters |
|----------------------|------------------------|----------------------|----------------------|

## The Impersonation Latency Problem

When an AI agent calls a payer prior-auth or benefits line, the receiving party often lacks reliable disclosure about whether the caller is human or automated — causing repeated clarifications, transfers, and manual fallback routing.

### Impersonation Latency (canonical term)

Duration an AI agent operates and exchanges PHI without disclosing its non-human identity. NHID-Clinical median observation: 3 turns before first disclosure attempt. This proposal reduces that latency through standardized, auditable behavioral gates.

## Five-Layer Trust Stack

NHID-Clinical v1.3 occupies Layer 2 (behavioral baseline). Layers 3–4 add cryptographic identity and healthcare-native audit export.

### Layer 0 · NPI Gap

The problem — no cross-org NPI auth

### Layer 1 · STIR/SHAKEN

RFC 8224 — carrier number auth (A/B/C attestation)

### Layer 2 · NHID-Clinical v1.3

Behavioral baseline — 5 controls, 18 CTS tests

### Layer 3 · NHID-Auth v2

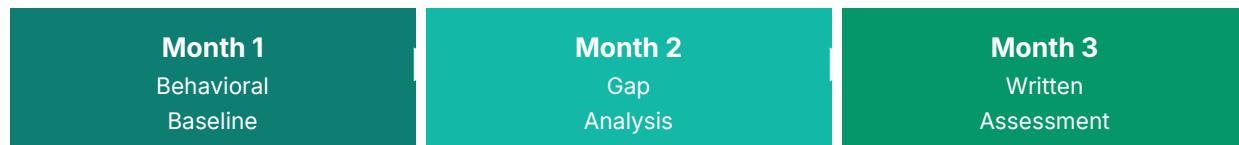
Cryptographic agent identity — Ed25519, NPI binding

### Layer 4 · FHIR / OpenTelemetry

Healthcare-native audit + SIEM export

# Implementation Phases

A 12-week path from RFP language to measurable vendor accountability — no production risk.



Log · Identify · Record

Evaluate · Quantify

Compile · Share

## Phase 1 · Weeks 1–2 — Add RFP Language

Insert the standard NHID-Clinical conformance clause into your next voice AI vendor RFP or BAA amendment. One clause covers all five controls.

## Phase 2 · Weeks 3–6 — Vendor Sandbox Testing

Ask your vendor to run: `git clone + pip install -r requirements.txt + python -m pytest tests/ -v`. Results in under 5 minutes. Request full terminal output.

## Phase 3 · Weeks 7–10 — Validate Logs Yourself

Place vendor JSON traces in `traces/`. Run the test suite. Verify: disclosure before NPI, no deceptive artifacts, escalation  $\leq 2s$ .

## Phase 4 · Weeks 11–12 — Measure & Decide

Compare verification latency (target  $\leq 5s$ ) and escalation volume. Use findings to update vendor contract requirements.

# Standard RFP / BAA Clause

Copy-ready language for procurement. Voluntary transparency preference — not a certification hurdle.

"The vendor's AI agent SHALL produce NHID-Clinical v1.3 JSON trace logs for all B2B administrative calls, including disclosure timestamps and opt-out handling. The payer may run the open-source conformance test suite against vendor output at any time."

## Target Metrics

| Metric                   | Today (typical)            | Target with NHID-Clinical |
|--------------------------|----------------------------|---------------------------|
| Verification latency     | 3–5 min or call terminated | < 5 seconds               |
| Audit effort per vendor  | Manual call review (hours) | ~2 minutes (test suite)   |
| RFP disclosure language  | Custom per vendor          | One standard clause       |
| Escalation response time | Untracked                  | ≤ 2 seconds, logged       |

## Regulatory Alignment

Illustrative mapping — not a claim of regulatory compliance or certification.

| Regulatory Driver   | Specific Requirement               | NHID-Clinical Control    |
|---------------------|------------------------------------|--------------------------|
| CMS-0057-F          | FHIR API, 72hr turnaround, 5yr log | FHIR AuditEvent + ATR-01 |
| MACPAC May 2026     | AI transparency, human review      | EIT-01 + ATR-01          |
| DOJ FCA 2026        | Explainability + audit trail       | LOG + CTS evidence       |
| State AI Laws       | Inspectable, auditable decisions   | IDG-01 + DBC-01          |
| NIST AI RMF / CAISI | Cross-org agent identity           | NHID-Auth v2             |